

CHAPTER 7

Setting the Table for Treatment

In our experience, all kinds of people can access phenomena in their internal world. Between us, we have worked successfully with clients from across the economic spectrum who arrived in therapy with a representative variety of diagnoses and ages, hailing from a broad (though by no means all-inclusive) range of cultures. We have found that most people have an intuition about the IFS process. Intellectual managers, however, are often reluctant to cede the spotlight. When they are psychologically sophisticated and have a habit of being in charge, they slow things down in IFS therapy rather than speeding it up. Fearful managers can also react negatively to IFS therapy. Our assignment is to accept these parts, tailor our language so that clients can understand IFS concepts, and be flexible about accepting the many different ways in which clients engage with their inner worlds.

WHO IS APPROPRIATE FOR IFS THERAPY?

Although we do not use every technique described in this book with every client, the IFS model of mind governs our understanding of human motivation and behavior, dictates our technical options, and guides every clinical choice we make. That said, both of us have caseloads that are skewed in the direction of trauma. As a result we have a lot of experience using IFS with people who carry multiple diagnoses and exhibit scary symptoms. In our experience, psychological processes that are considered pathological, including dissociation, flashbacks, various addictions, eating disorders, panic attacks, depression, anxiety, phobias, suicidal ideation, and brief reactive psychosis, are generally the activities of parts—and we can negotiate with parts. So we use IFS with all our clients,

as we illustrate in examples throughout this book. We trust that therapists who are new to IFS will develop their own style, and we encourage everyone to creatively import compatible techniques from years of practice and trainings.

Of course, therapists have a responsibility to use the tools presented here safely. When should we linger in the foyer, informing the client about our model of mind and talking to managers, but not pressing forward to their world? When is it safe to keep going? The following are some examples of circumstances that call for caution: (1) If a client is highly polarized and has a limited number of sessions, it may be unwise to venture beyond befriending and validating her protectors. Although good managed-care groups know that providing highly traumatized clients with as many sessions as they need is cost effective, managed care can radically limit an individual's access to ongoing therapy. (2) When a client's external environment is dangerous or intractable, or the client's position in his family or job allows little room for vulnerability, we work first to make that environment safer. If that's not possible, we are cautious about going to the client's exiles, and we keep a close eye on how relatives will respond to the client making changes. (3) Finally, if something about the client or her situation triggers the therapist's protectors, interfering over time with his ability to lead from the Self, then he should not attempt to guide the client inside.

In order to accompany our clients safely in their inner worlds, we need time along with a reasonable amount of external safety and access to our Selves. But even when one or more of these conditions is compromised or absent, we can still help clients listen to the fears of their managers, understand the reactivity and good intent of their firefighters, recognize the vulnerability that drives the behavior of these protectors, locate their Selves, and begin to relate to their whole inner systems differently. In other words, even if we cannot complete the job by witnessing and unburdening exiles, developing relationships with protectors can be empowering.

IFS WITH HIGHLY DISTURBED CLIENTS

When we present the IFS model of therapy, we are sometimes asked: *When you talk about parts with highly disturbed clients, aren't you running the risk of further fragmentation?* The answer is yes. But the danger does not come from inviting people to view themselves as multiple beings. Highly disturbed clients are well aware of their multiplicity and are usually relieved to hear from someone who does not view it as pathology. The problem is not that the psyche is plural, but that the person's

parts are so highly polarized. Intense polarizations preclude Self-leadership, which means clients with highly polarized systems have little experience of effective inner leadership or the sense of continuity and cohesion.

As we detail in [Chapter 8](#), two important IFS techniques are [in-sight](#) (when the client's Self communicates with parts internally) and direct access (when the therapist's Self communicates with the client's parts). When these techniques are used improperly, particularly when managers are disrespectfully pressed to give premature access to exiles, intense inner conflict and chronic dissociation can increase a client's feeling of fragmentation. This, in turn, can set off powerful firefighter reactions, more inner conflict, and further polarization—which is the feeling of being fragmented. However, when therapists follow the guidelines outlined in this book and in the IFS skills training manual (Anderson, Sweezy, & Schwartz, 2017), in-sight and direct access are generally safe techniques. We also strongly recommend Level One training (which also has a manual [Pastor & Gauvain, 2019]) with the IFS Institute (<https://www.IFS-institute.com>). When we help extreme, isolated, polarized parts to disarm and trust the Self, they integrate and harmonize, causing the feeling of fragmentation to decrease rather than increase.

IFS WITH CHILDREN AND ADOLESCENTS

Children are often more receptive to the IFS model than adults because they are less socialized away from their multiplicity. When I (RS) discovered that even young abused children have access to the Self when their parts separate, I finally became convinced that the Self exists fully formed in everyone and doesn't have to develop through a nurturing external relationship. Like all child therapists, IFS therapists who work with children often use external props such as a sand tray with little figures or hand puppets. They may also invite children to draw their parts and interact with the drawings, make up stories, or enact stories in psychodrama. Any strategy used in play therapy can be adapted to IFS with children or with adults, for that matter. Similarly, IFS therapists can use the model with adolescents. Once therapists make a connection with the parts who guard adolescents' self-images or who do not trust adults, teens are often adept with IFS. For more on using IFS with children, see Pamela Krause's chapter, "IFS with Children and Adolescents," in *Internal Family Systems Therapy: New Dimensions* (2013) and the books by Arthur Mones (2014) and Lisa Spiegel (2017).

IFS WITH GROUPS

Many clinicians who specialize in group therapy have adapted the IFS model, which is now used with a wide variety of group formats and client populations. One method is to work with a single client while the rest of the group watches. This process has several advantages: The client senses the Self energy of the group, which advances the work and helps that client's parts to feel highly witnessed; meanwhile, those who are watching the client often feel inspired to go to similar parts internally. In this way, the vulnerability of one client's work often stimulates rich inner work for witnesses, followed by open group discussion. Psychodrama is also easily adapted to IFS group therapy, with one member directing other members to take the roles of various parts and illustrate the interactions of an internal cluster of parts that include protectors and an exile.

INTRODUCING THE IDEA OF PARTS TO CLIENTS

When clients come to a first IFS therapy session, their managers will be assessing us—and our mode of therapy—to see if it is a safe and worthy investment. After hearing what brought the client to therapy, our first offer of help involves language. In our experience, the majority of clients come to therapy already speaking about their parts (e.g., *One part of me wants to go to my high school reunion, but another part is afraid!*), so the shift to talking about parts—and then with parts—comes easily. When we normalize the plural mind, we make room for clients to experience themselves as more than their feelings of neediness, incompetence, shame, and rage. And as clients discover that these tormenting feelings come from parts, often very young ones who need the very help the client can offer, their outlook on life begins to change for the better.

SHIFTING TO PARTS LANGUAGE

The following sections illustrate two ways of introducing the concept of parts and shifting into talking about (and then with) parts. The first is to restate the problem by calling the client's thoughts, feelings, and sensations *parts*. The second is to inquire about the client's inner self-talk and behavior toward others.

Restating the Problem

Following is an example of introducing the concept of parts by calling the client's thoughts, feelings, or sensations *parts*.

STELLA: I try not to purge and I succeed more often these days, but my parents see failure, not progress. The minute I think “I’m doing so great!” I picture them disapproving and all my pleasure disappears.

THERAPIST: And then what happens?

STELLA: I feel like “Why bother? I may as well purge.” But then again I think, “No, I should stop!”

THERAPIST: So two parts of you have an argument. One is hurt that your parents don’t see progress and tells you to give up, and the other pushes you to keep trying, no matter what. Is that right?

[Based on this clear description of a polarity, the therapist takes the opportunity to introduce two important concepts: that Stella has parts, and that her parts can argue with each other.]

STELLA: Yeah.

[In this context, the concept of parts is unremarkable to Stella.]

THERAPIST: We can help these parts if you want. But first, let me ask you a question. You saw an image of your parents being critical. Have they been critical?

STELLA: Well, sometimes. My doctor told them to stop getting mad at me, and I know they’re trying.

THERAPIST: So how about you? Do you have an inner critic?

[A critic with an agenda can interpret feedback from others as criticism, amplify actual criticism, or tell the client that others are critical without any evidence. The therapist is mindful that inner critics habitually interpret feedback from others in a negative way, whether or not the feedback was intended to be negative.]

STELLA: Oh, yeah! I feel like a loser all the time because I can’t stop.

THERAPIST: So when you purge, your critic calls you a loser?

STELLA: *(tearfully)* Yeah exactly.

THERAPIST: Which makes another part sad?

STELLA: This sad part has been around a lot lately.

[Note that Stella goes from accepting to using parts language in her last response.]

Inquiring How the Client Speaks to Herself and Acts toward Others

Next is an example of shifting to parts language by inquiring what the client says to herself and how she acts toward others. Mrs. Gorelick is Stella’s mother.

MRS. GORELICK: I can’t stand the mess Stella leaves in the bathroom. If she doesn’t stop this soon, I don’t know what I’ll do.

THERAPIST: What do you say to yourself when you find the mess in the bathroom?

[The therapist moves immediately to the client's inner conversation.]

MRS. GORELICK: I say she's doing this to rub my nose in it!

THERAPIST: And when this part tells you that Stella is rubbing your nose in it, how do you act toward Stella?

MRS. GORELICK: I yell and threaten to throw her out. And then I feel guilty.

[A polarity.]

THERAPIST: And what does the guilty part say to you?

MRS. GORELICK: Her problems are my fault, so I shouldn't get mad. I'm a bad mother ... but what else can I do?

THERAPIST: Shall I sum up? Okay. I'm hearing that one part gets furious with Stella, while another part criticizes you. Is that right?

MRS. GORELICK: Yeah. Pretty crazy, huh?

[At this point, Stella's mother simply experiences her inner conflict (the polarity) as a confusing loss of control.]

THERAPIST: No. Stella's problem is scary, and your reaction is normal. But even though inner disagreements are normal, this one gets in the way of your having a better relationship with Stella. Do you agree? If you want, we could make family sessions a twofer: Improve your relationship with Stella and help these parts of you, too.

[The therapist normalizes internal conflict, and then acts as a "hope merchant" for the client's system, offering help inside and out.]

MRS. GORELICK: What about her parts?

[And, naturally, Mrs. Gorelick's protectors remain guarded about being blamed.]

THERAPIST: Absolutely. Stella's parts too. Everyone has parts.

As these examples illustrate, the simple act of translating the client's speech into parts language is an initial step toward broadening her perspective. Since this broader perspective promises to be less shaming and more empowering (*I am more than my misbehaving or injured parts, and others are too!*), clients who come to therapy in the grips of a shaming manager or an angry firefighter often get enough space to feel curious just by thinking and speaking in terms of parts. In addition, parts language continually orients the therapist. When the client is able to attribute a feeling or thought to an entity (*A part of me feels*) rather than speaking of the feeling or thought as if it were the whole of the client's being (*I have a feeling*), we both stay aware of her parts and avoid stepping on their toes.

THE INTERNAL SYSTEMS OF PEOPLE WHO SEEK THERAPY

Most people come to therapy in the grip of inner conflict and negative feelings such as fear, anger, grief, shame, or maladaptive guilt. From an IFS perspective, all parts in the triangle of manager, exile, and firefighter are hurt, frightened, and frozen in the past. They have been doing their best to cope with life's challenges. We see their relationships as falling into one of two categories: (1) protective (e.g., a manager or firefighter protects an exile) or (2) polarized, in which two protective parts are in conflict with each other. We explore their relationships, but not to resolve their disagreements or interfere with their alliances. Instead, we aim to introduce a game changer: the client's Self. [Box 7.1](#) summarizes typical features of the internal systems of those who seek therapy.

BOX 7.1. The Inner Relationships of People Who Seek Therapy: Parts Are Either Protective or Polarized

Protective: We call proactive parts who try to inhibit exiles from reaching consciousness *managers*. These protectors can be harsh, smothering, or kindly. In contrast, reactive protectors, whom we call *firefighters*, try to suppress, numb, or distract from exiles after they reach consciousness. Firefighters tend to use anything that will stimulate or soothe quickly, including food, drugs, alcohol, self-harm, rage, and sex.

Polarized: Managers often polarize with firefighters (e.g., the caretaker vs. the angry part), but they can also polarize with each other (e.g., the caretaker vs. the workaholic). In contrast, firefighters are less likely to polarize with each other than they are to “pinch-hit” for each other (*When I stopped bingeing and purging, I started drinking too much*).

ASSESSMENT IN IFS

An IFS assessment generally revolves around two questions. First, we want to know if the symptomatic part (e.g., depression or anxiety) is a protector or an exile. Second, we want to know how much access the client has to the Self.

The First Assessment Question

Let's begin with the first question. The easiest way to find out about a part is to ask it. For example, *What do you do for Sasha?*

The Part Who Is Doing a Job

If a part endorses the idea that it is *doing* a job, we know a few things right off the bat. Its role is protective and it is either proactive (a manager) in suppressing emotional pain or reactive to emotional pain (a firefighter). Managers are often motivated by fear to inhibit other parts. They try to inhibit feelings they believe are too much, like shamefulness, fear, sadness, guilt, pain, and anger. And they are generally polarized with at least one other protector, either another manager or a firefighter, which motivates them to intensify their behavior. In addition, they are not capable of healing injured exiles. Most managers will not change in any fundamental, permanent way until we help the exile they protect to leave the past and let go of burdens. Reactive protectors—firefighters—are at the other end of the cycle. They are reactive rather than proactive; they distract from overwhelming feelings that have already arisen. Otherwise, however, they too are motivated by fear of pain and are usually polarized with at least one other part (often a manager). They too cannot heal other parts. And they too are unlikely to change until the exile they protect is unburdened.

The knowledge that most protectors will not change their behavior until the part they protect is healed could improve many aspects of psychotherapy and improve our culture's tolerance for protective parts in general. The expectation that people should be able to use willpower, for example, to stop addictions, control anger, or override fear only exacerbates inner polarizations. It also exacerbates addictions, fills our prisons, and drives our massive consumption of medications. This worship of willpower comes from the legacy burden of individualism, which permeates our culture and us. Most protectors will gladly stop their behavior once the part they protect has been healed and the system is less vulnerable. Until then, they will continue to believe that anyone who asks them to stop is unaware of the dangers of stopping, and they will fight to do their jobs—often as if the client's life depended on it, since the client's life often did depend on this protector stepping in when she was young. People who do succeed in controlling firefighter behaviors with willpower are relying on managers to sit on their firefighters as well as their exiles, which makes for a very tense, vulnerable recovery, like a dry drunk. [Box 7.2](#) summarizes the typical characteristics of protectors.

BOX 7.2. A Part with a Job Is a Protector

- # A part with a job is either a manager or a firefighter.
- # If it is a manager, it will focus on inhibiting the feelings it believes are dangerous, especially shame, fear, sadness, guilt, pain, and anger; if it is a firefighter, it will focus on distracting from negative feelings.
- # Both managers and firefighters are motivated by fear.

- # Both managers and firefighters tend to polarize with at least one other protective part, which motivates both sides to continually escalate their behavior.
- # Neither managers nor firefighters can heal the injuries of exiles.
- # No protector will change in any fundamental, permanent way until the injured part it protects is helped to leave the past and let go of its burdens.

The Part Who Is Not Doing a Job

The target part who says it is not doing a job is likely to be an exile—a part who got scared or shamed and then banished from consciousness and the flow of time, leaving it feeling alone and abandoned. These parts long to be rescued and redeemed. If they are sufficiently triggered, they are capable of breaking out of exile and overwhelming the client with their feelings. An exile jailbreak often evokes extreme firefighters such as dissociation, addiction, self-harm, and suicide. On the other hand, and this is key to IFS therapy, exiles are also capable of not overwhelming the client if they are convinced that they can get attention by staying separate. An exile's ability to exercise choice about emotional overwhelm is vital to IFS therapy. When an exile agrees not to overwhelm the client, and the client's Self stays present, protectors gain confidence and cooperate, paving the way for our ultimate goal of witnessing and unburdening the exile.

Because any part can control the level at which it blends with the Self, we are able to work with very delicate inner systems largely without having to use the grounding techniques and affect regulation skills that are a prevalent feature of most trauma therapies. From the IFS perspective there are several problems with focusing on building such skills in trauma therapy. First, if a client has a panic attack or dissociates in a session and we tell her to look into our eyes, feel her feet on the floor, and breathe deeply, we are signaling the part who has come forward that we need it to leave because it is too scary or dangerous. This is the last thing we want to convey to frightened, desperate parts. Therefore, in IFS when a client begins to be symptomatic in session, we notice the part and talk about it with the client's Self or ask to talk to it directly. Once we learn why it is taking her out, we negotiate with it to allow her to return. Then, suddenly, the client is grounded again. If, for example, a client has a panic attack we say something like, "I see that a frightened part has taken over. It is very welcome here. I get that it's stuck in a scary place in the past, and we're going to get it out of there. It would help a lot if it were willing to separate from you just a little so you can be with it rather than being overwhelmed by it." If the part trusts that we mean what we say, the client will suddenly be grounded again (i.e., the Self will be available), and she will be able to help the scared part. Most exiles overwhelm because they are afraid of being locked away again if they back off.

Once they understand that our goal is the opposite—to pay attention and help—they usually agree to separate and stop overwhelming. [Box 7.3](#) summarizes the typical characteristics of exiles.

BOX 7.3. A Part without a Job Is an Exile

- # The part got hurt and banished, and continues to feel unbearably lonely and worthless.
- # The part wants to be rescued and redeemed.
- # The part is capable of overwhelming the client with feelings that evoke extreme reactions from protectors, such as dissociation, addiction, self-harm, and suicide.
- # The part is also capable of not overwhelming the client if it believes that differentiating is the best way to get its needs met.
- # Since emotional overwhelm is the foremost fear of protectors, an exile's ability to decide not to overwhelm is vital to the process of IFS therapy. When the exile agrees not to overwhelm, protectors are more likely to cooperate, which paves the way for witnessing and unburdening.

The Second Assessment Question

Once we learn whether a part is doing a job or not (i.e., whether it is a protector or an exile), we have a second important assessment question: How much access does the client have to the Self? This we gauge by noticing if he can speak for his parts rather than speaking from them, if he can be interested in his parts, and if he is able to get to—or imagine getting to—a feeling of curiosity, kindness, or compassion toward his parts. As long as the client is curious about his parts, we can use the communication method called *in-sight*. In-sight involves the client's Self communicating internally with the client's parts. But if the client is jittery, tearful, self-critical, angry, confused, sleepy, dissociative, or in some other way extreme, we can expect to use a good deal of direct access, in which the therapist's Self talks with the client's parts. In this case the therapy will probably take longer.

WHO IS RUNNING THIS CLIENT'S LIFE?

As we ponder a client's behavior, we ask ourselves who is dominant in her system. An exile? For example, the client is unable to hold a job, is overwhelmed by flashbacks and panic attacks, is frequently in the hospital, or is in a violent relationship and will not leave. Or, perhaps a manager is in the driver's seat? For example, the client is able to work and can function well enough, but is critical of herself and others; or she is rigid and wary; or she is chronically depressed and has

breakthrough anxiety. On the other hand, perhaps a firefighter is in the driver's seat. For example, the client is abusing fentanyl, overexercising, or bingeing and purging. Finally, the client may be dominated by a polarity between two protectors. For example, the client veers between being an attentive spouse and mother at home and having affairs at work. Parts that can dominate a system are summarized in [Box 7.4](#).

BOX 7.4. Which Part(s) Dominates the Client's System?

1. An exile?
2. A manager?
3. A firefighter?
4. A polarity between two protectors?

BE CURIOUS ABOUT A PART'S ROLE IN THE SYSTEM

We find it helpful to know the role of any given part in the client's system because different roles call for different approaches. Managers are proactive and inhibitory. They do not want exiled feelings to reach consciousness. They want to keep the client functioning, they care about consequences, and they can be harshly critical, but they will admit to being tired and having failed. We help managers by appreciating their desire for stability, validating their fear of firefighters, and offering that the client's Self can negotiate with firefighters and take care of exiles.

In contrast, firefighters are reactive and often (though not always) disinhibiting. They want to distract from emotional pain and will do so in any way possible as long it works quickly and is effective. The horizon is always close for firefighters; they profess to being unmoved by even devastating consequences and are reluctant to admit failure. As a result, motivating a firefighter to cooperate can appear daunting. To be persuasive with a firefighter, keep in mind that they care about maintaining their ability to protect and they want to be appreciated. We therefore tell firefighters, *You are the boss*. We also appreciate their sacrifices and validate their positive intention for the client. While avoiding scolding these parts, we offer to help them get relief from burdensome jobs and we help protect them from other parts (and people) who shame, damn, or want to kill them off. Not least, we also offer them the prospect of a preferable role.

In contrast to busy protectors (both managers and firefighters) who talk about what they can, should, will or will not *do*, exiles simply exist in painful, lonely, often dangerous places. Some hide, some struggle to be seen and rescued, some are buried so deep in the body or in a dungeon that the

client is unaware of their existence. We help exiles by witnessing the bad things that happened and caused them to believe they were bad. When a part is witnessed (understood, validated, loved) by the Self, it goes from believing *I am bad* (worthless, unlovable, defective, ugly, etc.) to feeling accepted and understanding that *a bad thing happened to me*, at which point it is usually ready to let go of its burdens.

TRANSLATING DSM DIAGNOSES INTO IFS

As we hope the description above makes clear, assessment and diagnosis in IFS bear little resemblance to standard assessment and diagnosis based on any iteration of the DSM. In IFS we conceptualize symptomatic behavior as the product of a plural mind, and for both physical and mental ailments we assess connections between mind, brain, and body (Anderson et al., 2017). We view DSM diagnoses as descriptions of various activated parts whose behaviors were intended to ensure survival by coping with past danger, preventing future danger, and maintaining inner balance. To say protective parts have positive intent is not to say that their actions have positive consequences, far from it. But regardless of the effects of their behavior, we must understand their positive intentions if we are to help them. We can take any diagnosis in the DSM and provide an alternative, nonpathologizing view of that condition, based on which parts dominate in any given client.

WHAT DO WE WANT TO CHANGE IN IFS THERAPY?

Some models of therapy are based on the principle of addition. On top of new insight and understanding, the therapist helps the client add new skills, experiences, and ways of relating with external others. We appreciate the value of practicing new skills and having new experiences, but we view their acquisition as the natural outcome of the client releasing constraints and moving forward in life, which is our priority in IFS therapy. Negative self-referential beliefs (*I am worthless*) are profoundly inhibiting, as are many of the feelings engendered by traumatic experience (*I am terrified, I am enraged*). Inhibitions of this nature lead to all kinds of consequential, negative sequelae. In IFS we aim to release our clients from constraints so that they can be who they truly are in the present, free of burdens from the past.

Along these lines, there are three primary goals for using IFS with individuals: (1) Liberating parts from extreme roles so they can move on to preferred, valuable roles; (2) restoring the trust of parts in the leadership of the Self; and (3) re-harmonizing the system of parts such that they get to

know each other and form productive collaborations. As these goals are achieved, people feel more unitary, but their parts still exist. The change is that their parts are now living in synchrony and no longer stand out. The reader may have noticed that these same goals apply to human systems at all levels, which points to the many parallels between internal and external systems. Our motto, therefore, is that *all parts are welcome*. If clients start therapy feeling they have too little space for all inner inhabitants, we aim for therapy to end on the opposite note, with their parts harmoniously embodied because the client's inner environment has become spacious, inclusive, and Self-led.

In IFS we also want clients to change their relationship with time. Protective parts use the memories of the past in various ways to manage feelings in the present: discouraging in order to inhibit hope, disappointment, and suicide; denying to try to avoid consequences; and using all kinds of strategies to avoid acceptance and grief, including undoing (*Let's go back to before that happened and erase it*), speculating (*What if I had delayed by just 5 minutes?*), reminiscing nostalgically (*Things were better when ...*), minimizing (*That wasn't so bad*), and comparing (*Others have it worse/better*). Protectors also make use of the future: They fantasize to distract from reality (*I will be the winner!*) and escape (*Imagine being in that water right now!*); they predict to exert control (*If you commit to that relationship, you'll end up with someone just like your father*) or inhibit (*That airplane is going to fall out of the sky!*) or steer (*Wouldn't it be nice to be rich?*). If body-based addictive strategies (food, exercise, drugs, alcohol, pornography) are firefighter strategies for soothing and distracting from emotional pain (Catanzaro, 2016; Sykes, 2017; Wonder, 2013), undoing the past and imagining or predicting a particular future are manager strategies for controlling feelings. Although their efforts produce a desired effect in the short term (or they wouldn't keep doing it), there are inevitable long-term costs to managing feelings by fantasizing and denying reality.

While protectors are busy moving around in time to manipulate memory and expectation, exiles are trapped in time. Once the client's Self has established a trusting relationship with an exile, it takes the Self on a tour of the past, showing where and how it got stuck. This tour gives the Self the opportunity to validate the exile's perspective (*You were hurt*) and to reassure it (*You didn't deserve that*), until the part is ready to let go of the beliefs and feelings—the burdens—that accrued from those experiences. To conceptualize time in IFS therapy, therefore, it helps to think literally. We notice protectors playing tricks with time, and we accompany exiles back in time. Our ultimate aim is for clients to be alert and available in the present, to remember and visit the past if and when they want, and to move into the future imaginatively, free of unnecessary constraints and confident that their concerns, when they have them, will be based in reality. The burdens of parts who are stuck in

the past are like grains of sand in the works of a clock. Until we release them, time cannot go forward.

A BRIEF SUMMARY OF IFS TREATMENT

When a client describes her problem we ask questions about her inner experience around the problem and feed back what we hear, adding the phrase, “So one part of you ... and another part....” Once the client has identified a few parts, we ask which one she wants to help first and if it seems like a protector we ask if any other parts have concerns; then we help any reactive parts and proceed to get to know the target part. If the target seems like an exile, however, we tread more cautiously, first addressing the problem of exile overwhelm and any protector fears.

Throughout this process we look for the client’s inner polarities, which can involve a manager and a firefighter or a manager and a manager, keeping in mind the essential difference in the roles of managers and firefighters: Managers tend to be preemptive and controlling in their efforts to keep exiles from activating; firefighters jump into action after an exile is activated, either reactively and impulsively distracting from or dousing the flames of exiled emotion.

As they take action, protective parts have a few common concerns. They fear each other (*she would be too angry, he would be too passive*) or they fear exiles (*she would be overwhelmed by sadness, he would know he’s unlovable*). Additionally, they are often concerned about their own fate (*If I don’t have this job, will I disappear?*). As we get to know these parts, we can ask them what would happen if they stopped doing their job. The answer tells us if their first fear involves a polarized protector or a fragile exile. We address their fears by proposing that the client’s Self can take care of the exile or help that polarized protector de-escalate, and reassuring them that they will be free to choose a new role.

Once protectors give permission for the client’s Self to help an exile (and the timeline for this will range widely), the Self can form a trusting relationship with the exile and ask what it needs. Most exiles need the client’s Self to witness burdening experiences from the past. In this case, the Self accompanies the exile to the past and, after the exile feels fully witnessed, the Self offers to do whatever the part needed someone to do at the time (a *do-over*, which is an emotionally corrective experience). Finally, when the exile is ready, the Self brings it out of the past and invites it to let go of its burdens. During witnessing and unburdening, the exile is in charge of all decisions. After the exile unburdens and invites in the qualities it will need going forward, the Self returns to protectors to see if they are ready to take new jobs. [Box 7.5](#) lists key steps in IFS Therapy.

BOX 7.5. Key Steps in IFS Therapy

FINDING A TARGET PART: START WITH PROTECTORS

1. Ask the client, "What brings you to therapy?"
2. Ask the client, "Can we talk about parts?"
3. Find a target part, preferably a manager, or focus on a polarity between two protective parts.
4. If an exile comes up first, ask it to let the client's Self negotiate first with protectors then return to locating a protector who will be the target part.
5. Get permission (from all other parts) to talk to the target part.
6. Ask the target part about its job.
7. Ask the target part about its fears.
8. Offer to introduce the target part to the client's Self.
9. Ask for permission to help the exile.
10. If the target part agrees, check to see if any other parts object. If yes, go through the same steps with those parts. If no, proceed to the exile.

WITNESSING AND UNBURDENING: EXILES AS TARGET PARTS

1. The exile meets the client's Self.
2. The exile tells the Self what it needs.
3. The Self witnesses the exile's burdening experiences.
4. The exile lets its burdens go.
5. The exile invites in the qualities it will need going forward, in place of burdens.
6. The Self checks back with protectors and invites them to find new roles.

CONCLUSION

People generally come to therapy because they fear their feelings and believe they are unlovable. In IFS we do not view these symptoms as signs of pathology. For us, fear, burdensome beliefs, inner conflict, and extreme behaviors are a product of systemic imbalances and constraints that often have roots in the events of childhood. Additionally, by *systemic* we mean imbalances and constraints that can develop at any level among multiple nesting systems, starting with an inner system of psychic parts, which is nested in a family system, which in turn nests in a community, which is nested in a culture and a civic structure. Given our extreme childhood vulnerability and the systemic complexity of our lives, most of us accumulate at least some fear and self-doubt, which means we have parts who need to be liberated.

In IFS we aim to liberate parts and rebalance systems by releasing constraints. We begin in individual therapy by being curious about the behaviors and motivations of protective parts. During early sessions we are most interested in the relationships within the client's inner system: who is currently doing what to whom and why. Once protectors feel sufficiently understood and appreciated by the Self, they will allow access to the exile. After this we move into the second phase of treatment, which often moves along far more rapidly and is devoted to the Self witnessing and unburdening the exile, who is usually stuck in the past and has an important story to tell. If the client's system has many exiles, this process may be repeated multiple times with different clusters of parts. Every time an exile tells its story, lets go of its burdens, and the client is reunited with her natural self-righting abilities—her Self—inner space and time expand.